

METABOLAE

Ceylon Cinnamon 7200mg

THE D.R.I.V.E. STRATEGY DOCUMENT

Discover. Reframe. Ignite. Variate. Evolve.

The complete creative strategy system for scaling Metabolae into the dominant Ceylon cinnamon brand in the metabolic health category.

Built to scale. Built to compound. Built to last.

Executive Summary

This is not a creative brief. This is the operating system for every ad, every angle, every variant, every batch, and every test Metabolae will run for the next twelve months. Built using the DRIVE framework against the analytical brief, the live competitive landscape, the existing winning ads already running for this brand, and the foundational principles of Schwartz, Halbert, RMBC, and direct response.

The objective is not a single hero ad. The objective is a creative production system that compounds. One that produces a Big Idea strong enough to anchor twelve months of campaigns, an angle library deep enough to feed the Andromeda algorithm, a mechanism story credible enough to convert skeptics, and a learning loop fast enough to widen our lead every cycle.

The strategic situation in one paragraph

Metabolae sells a commodity formulation in a saturated category that has reached Stage 4 of market sophistication. Reddit users are openly mocking the lookalike packaging. Multiple competitors run identical "7200mg + MCT oil" claims. Amazon is flooded with 4-star alternatives at lower prices. The category-level promise is dead. The mechanism claims are dead. We cannot win on "Ceylon cinnamon supports blood sugar." We cannot win on "7200mg equivalent." We cannot win on "MCT for absorption." Every brand says this. Every brand sounds the same. The only path forward is a reframe sharp enough to reset the category conversation and an identity layer strong enough to make Metabolae the brand chosen by people who already know cinnamon doesn't work.

The Big Idea this document orbits

Cinnamon is the most lied-to ingredient in the supplement aisle. The capsules don't work because they're not Ceylon, they're not concentrated to clinical levels, and they're sold as dry powder when the active compound only absorbs in fat. Most people who tried cinnamon and gave up didn't fail. They were sold the wrong cinnamon, in the wrong dose, in the wrong form. Metabolae is the corrected version of the supplement they already tried.

This positioning does five things at once. It validates the prospect's prior failure (which is empathy, not blame). It indicts the entire category (which creates contrast). It makes the claim believable through specificity (Cassia versus Ceylon, powder versus extract, dry versus oil). It positions Metabolae as the only logical choice for someone who actually wants the result. And it gives us infinite angle territory because every angle becomes a different way of saying "the cinnamon you tried was lying to you."

What this document delivers

- **DISCOVER.** The mass desire driving the category. Sophistication map by ICP. Three primary ICPs and three underserved ones the category is sleeping on. The Big Idea and its proof architecture.

- **REFRAME.** The four-line mechanism chain that converts skeptics. The full Cassia-versus-Ceylon contrast. The dose-equivalency story. The MCT bioavailability lever. Visual proof for every claim.
- **IGNITE.** Per-ICP emotional driver maps. The persuasion stack adapted for short-form, mid-form, and VSL. Conversion amplifiers, offer architecture, and the role of humour for ego defense.
- **VARIATE.** Twelve fully-built angles, each with format assignments, ICP targeting, sophistication match, and production briefs with non-negotiables marked. Built to feed Andromeda properly.
- **EVOLVE.** The diagnostic for reading data. The pattern extraction questions for compounding learning. The four-week workflow that turns this document into a machine.

Compliance posture

Per direction, copy is written at maximum strength. Risk levels are flagged inline so legal review can soften specific lines without rewriting the strategy. The architecture stays compliant even at full force because the core reframe is about ingredient quality and dose adequacy, not therapeutic effect. The strongest claims live in testimonial frames, which carry different exposure than brand voice claims.

PART ONE

DISCOVER

Find the desire. Map the market. Write the Big Idea.

1. The Mass Desire

The category sells cinnamon. The market is buying something else entirely.

Strip away the marketing language and the ingredient claims and what every Metabolae buyer actually wants comes down to one thing: control over a body that has started to feel like the enemy. Not blood sugar. Not metabolism. Control. The buyer has watched their numbers creep up, watched their energy crash, watched their sleep break, watched their pants get tighter, and watched their doctor's face change when she pulls up the chart. They feel betrayed by their own biology and they want it back.

Cinnamon is just the vehicle. The real product is permission to feel like themselves again. To eat dinner without dread. To wake up without panic. To stop being the family member everyone is worried about. To stop watching their own future contract.

THE MASS DESIRE: To stop being afraid of their own body. To regain authority over a system that has started making decisions without them.

This is bigger than cinnamon. It is bigger than blood sugar. It is the operating emotion behind every metabolic health purchase in the over-45 demographic. Every angle in this document reconnects to it.

2. The Sophistication Map

Markets are not monolithic. Different segments of the cinnamon-buying audience are at different stages of sophistication. Running one stage of copy is leaving 60% of the addressable market on the table. Here is how the category breaks down right now:

Stage	Who's at this stage	What kills them	What converts them
Stage 2 — Bigger claims	Newly diagnosed prediabetics. Just got the lab result back. Still	Generic 'supports blood sugar' copy.	Specificity. Numbers. The biggest, sharpest, most concrete claim about

Stage	Who's at this stage	What kills them	What converts them
	researching. Have not bought a cinnamon supplement yet.	Sounds like every other ad.	what cinnamon can actually do — backed by a credible source.
Stage 3 — New mechanism	Tried generic cinnamon. Saw nothing. Wrote off the ingredient. Now scrolling skeptically.	More claims. They have heard them all and they did nothing.	A mechanism that explains why their previous cinnamon failed. Cassia vs Ceylon. Dose gap. Powder vs oil suspension. The reframe IS the sale.
Stage 4 — Mechanism amplified	Have heard 'Ceylon vs Cassia.' Have seen '7200mg.' Have seen MCT claims. Saturated by lookalikes.	Repeating what every competitor says. Indistinguishable.	Amplification. Specificity that makes the mechanism undeniable: 12:1 extract ratio, DNA verification, batch numbers, lab certificates, founder story, sourcing depth.
Stage 5 — Identity	Burned by every supplement. Don't believe ingredient claims anymore. Buy based on tribe, identity, founder trust.	Any health claim. They've heard it all.	Stop selling the product. Sell who they become by taking it. The nurse who refused metformin. The grandfather who got his appetite back. Identity, not persuasion.

The strategic implication

Most Metabolae competitors are running at Stage 2 ("Supports blood sugar! 7200mg!"). The winning ads already running for Metabolae are operating at Stage 3-4 (the Cassia/Ceylon reframe, the dose specificity, the MCT mechanism amplification). The biggest open territory in this market is Stage 5 identity work, which almost nobody is doing well. Our angle library will hit all four stages, with Stage 3-5 being where the long-term competitive moat gets built.

3. The Dominant LF8 Triggers

Every LF8 driver maps onto this category. The skill is knowing which one leads, which supports, and which to use when.

LF8 Driver	Role in this category	How it shows up in the work
Survival / Life Extension	PRIMARY	The bedrock fear: amputations, dialysis, blindness, nerve damage, kidney failure. Not

LF8 Driver	Role in this category	How it shows up in the work
		used as scare tactics, used as the unspoken context every prediabetic already lives inside.
Freedom from Fear	PRIMARY	Relief is the actual product. Relief from finger pricks that always disappoint. Relief from the doctor's tone changing. Relief from worrying every meal will hurt them.
Care for Loved Ones	SECONDARY	'My husband watches my plate.' 'My daughter is scared for me.' Not buying for self-improvement. Buying to stop being the source of their family's worry.
Comfortable Living	SECONDARY	Sleeping through the night. Not crashing at 2pm. Not waking to pee three times. The product that gives them their day back.
Social Approval	TERTIARY	Quietly powerful. Not wanting to be 'the one with diabetes' at family dinners. Not wanting visible weight gain to give away their condition. Underutilized in the category.
Superiority / Winning	UNDERSERVED ANGLE	'I beat my numbers without the medication my doctor said I'd need.' Pride at outsmarting the system. Massive opening for identity-driven copy.

STRATEGIC NOTE: The two LF8s nobody in the category is properly weaponizing are Care for Loved Ones (the spouse-watching-the-plate angle) and Superiority/Winning (the 'I beat the doctor' angle). Several of our angles below are built specifically around these underserved emotional doorways.

4. ICP Map: Three Obvious, Three Underserved

The brief identified three audiences. The brief was being polite. The actual category has at least six addressable ICPs, and three of them are being completely ignored by every competitor running ads in this space right now. The brand that builds creative for the underserved three will own this category in 18 months.

THE THREE OBVIOUS ICPs

ICP 1: The Newly-Scared Prediabetic (45-60, female-skewed)

- **Trigger event:** Doctor used the word 'prediabetic' for the first time in the last 90 days. A1C between 5.7 and 6.4. Was not expecting it.
- **Mental state:** Quietly terrified. Has googled prediabetes 40 times. Has not told her friends. Wakes up at 3am scrolling food blogs.
- **What they've tried:** Cutting carbs. Walking. Apple cider vinegar. A cinnamon supplement from the grocery store that did nothing.
- **Awareness stage:** Solution-aware. Knows cinnamon is a thing. Doesn't know which cinnamon.
- **Sophistication stage:** Stage 2-3. Has heard claims, beginning to discount them.
- **Dominant LF8:** Freedom from Fear (#1) + Survival (#2).
- **Belief blocking sale:** "I tried cinnamon. It didn't work. This is just more of the same."
- **Emotional doorway:** Validation of failure plus a credible explanation for why.

ICP 2: The Type-2 Refuser (50-65, male-skewed)

- **Trigger event:** Doctor mentioned metformin. Patient has refused, hesitated, or quietly thrown the prescription away.
- **Mental state:** Resentful. Distrustful of medication. Has watched a parent or sibling deteriorate on diabetic meds. Determined to find another way.
- **What they've tried:** Keto, intermittent fasting, berberine, ACV, cinnamon capsules. Some progress, not enough.
- **Awareness stage:** Solution-aware leaning toward product-aware.
- **Sophistication stage:** Stage 3-4. Has heard the claims, has tried mechanism-based products, looking for the one that proves out.
- **Dominant LF8:** Superiority/Winning (the desire to beat the system) + Survival.
- **Belief blocking sale:** "Every supplement I've tried promises results and delivers placebo. Why would this be different?"
- **Emotional doorway:** An insider/credentialed voice (nurse, doctor, biochemist) saying "here's what the others got wrong."

ICP 3: The Optimization-Minded Healthy Adult (35-55)

- **Trigger event:** Read about CGMs on a longevity podcast. Got curious about post-meal glucose spikes. Numbers are 'fine' but trending wrong.
- **Mental state:** Curious. Data-driven. Reads supplement studies. Suspicious of marketing.
- **What they've tried:** Already takes 8-12 supplements. Berberine, magnesium, cinnamon, chromium.
- **Awareness stage:** Most aware. Knows the science. Knows the actives. Discriminating between brands.
- **Sophistication stage:** Stage 4-5. Skeptical of all marketing. Buys based on transparency, sourcing, lab data.
- **Dominant LF8:** Survival/Life Extension + Superiority (longevity-as-status).
- **Belief blocking sale:** "I already know cinnamon works. Just show me your COA and your sourcing and stop selling me."
- **Emotional doorway:** Anti-marketing, transparent, founder-voice. Receipts not promises.

THE THREE UNDERSERVED ICPs THE CATEGORY IS SLEEPING ON

This is where the disproportionate upside lives. Almost no competitor is making creative specifically for these segments. Whoever owns them owns the next two years of category growth.

ICP 4 (UNDERSERVED): The Caregiver Buyer

This is the daughter, the wife, the husband, the adult child of a person with metabolic decline. They are not the patient. They are the one buying the supplement on behalf of someone they love who has either given up or is too proud to act.

- **Trigger event:** Watched their parent or spouse decline. Caught a moment that scared them: a fall, a hospital visit, a diagnosis update.
- **Mental state:** Burdened. Researching for someone else. Often shopping at 11pm after a worrying phone call. Willing to spend more if they believe it works.
- **What they've tried:** Tried buying their parent fitness equipment, glucose monitors, healthier groceries. Most got ignored.
- **Awareness stage:** Problem aware on behalf of someone else.
- **Sophistication stage:** Variable. Treat like Stage 2-3.
- **Dominant LF8:** Care for Loved Ones (overwhelmingly). Freedom from Fear (theirs, not the patient's).
- **Belief blocking sale:** "They won't take it. They never take anything. This will sit on the counter."
- **Emotional doorway:** A product that's easy enough that even a stubborn parent will take it. One softgel. No mixing. No taste. The hardest part of caregiving is compliance — Metabolae solves it by being effortless.
- **WHY THIS IS UNDERSERVED:** Every competitor speaks to the patient. Nobody speaks to the family member who's actually doing the research, holding the credit card, and shipping the bottle. This is a 30-40% addressable segment that nobody is targeting.

ICP 5 (UNDERSERVED): The Post-Pregnancy Glucose Scare (35-45, women)

Women who had gestational diabetes during pregnancy and were told they have a 50% chance of developing Type 2 within ten years. Now their kids are 5-10 years old, they're in their late 30s or early 40s, and they just got their first slightly-elevated A1C.

- **Trigger event:** First post-pregnancy A1C creep. Or a routine physical that revealed insulin resistance. Memory of GD pregnancy resurfaces.
- **Mental state:** Quietly panicked. Mid-life, busy, exhausted. Not the typical 'older prediabetic' demographic that brands market to.
- **What they've tried:** Trying to lose the last 15 pounds from pregnancy. Maybe a continuous glucose monitor briefly. Some berberine.
- **Awareness stage:** Solution aware.
- **Sophistication stage:** Stage 3.
- **Dominant LF8:** Care for Loved Ones (their kids will grow up watching them struggle if they don't fix this) + Survival.
- **Belief blocking sale:** "I'm too young to be one of those people. I shouldn't need a supplement."
- **Emotional doorway:** Speak to identity. "You are not your mother. You can intervene at the early signal instead of waiting for the late one."
- **WHY THIS IS UNDERSERVED:** Every cinnamon ad targets women 55+. The 35-45 GD-history segment is invisible to category marketers but they have higher LTV, higher AOV, and they buy on subscription. They are also active on social and high-intent on Instagram and Facebook.

ICP 6 (UNDERSERVED): The Ozempic Off-Ramper

People who went on a GLP-1 (Ozempic, Wegovy, Mounjaro) for weight loss or diabetes, lost the weight, and are now coming off the medication. They are terrified of regaining and looking for a maintenance protocol that doesn't involve another drug. This segment did not exist in 2023. By 2026 it is one of the largest emerging health-product audiences in the world.

- **Trigger event:** Hit goal weight on GLP-1. Insurance stopped covering. Side effects became intolerable. Doctor recommended off-ramping.
- **Mental state:** Anxious. Suspicious of regain. Hyperaware of food, sugar, and metabolism in ways they never were before. Will spend significant money to maintain results.
- **What they've tried:** The drug worked. Now they're scared of stopping. Looking for natural maintenance stack.
- **Awareness stage:** Most aware. Just exited a clinical environment. Sophisticated buyer.
- **Sophistication stage:** Stage 4-5.
- **Dominant LF8:** Survival + Social Approval (don't want to regain visible weight).
- **Belief blocking sale:** "The drug worked. Cinnamon won't be enough. I need a real solution."

- **Emotional doorway:** Position cinnamon not as 'the answer' but as 'the foundation of your post-Ozempic protocol.' Stack messaging. Sophistication respect. Don't promise results, promise infrastructure.
- **WHY THIS IS UNDERSERVED:** Massive segment. High AOV. Subscription-prone. Not a single Ceylon cinnamon brand on Amazon or Meta is targeting them with bespoke creative. Whoever moves first claims this segment for the next three years.

5. The Big Idea

Every other deliverable in this document orbits this single concept. If we can get the prospect to internalize this one idea, the sale becomes the only logical conclusion. If we fail to get it across, no amount of clever copy will save us.

BIG IDEA: The cinnamon supplement industry has been quietly selling counterfeit cinnamon to millions of people for thirty years. What's labeled "cinnamon" in 90% of capsules is Cassia, a different species with up to 250 times more coumarin and a fraction of the active mechanism. The clinical studies that made cinnamon famous used Ceylon. The capsules on the shelf use Cassia. Same word. Different plant. Different result. Metabolae is the corrected version.

Why this Big Idea works

- **It explains failure.** If the prospect tried cinnamon and saw nothing, they were not weak. They were lied to. This is empathy without pity.
- **It indicts the category.** Without naming a single competitor, every competitor's product becomes suspect. The prospect now requires proof of Ceylon sourcing from any brand they consider.
- **It is verifiable.** Anyone can google 'Cassia vs Ceylon coumarin.' The Wikipedia page agrees with us. The FDA agrees with us. The clinical literature agrees with us. The truth is doing the heavy lifting.
- **It opens infinite angle territory.** Every angle becomes a different way of telling the same fundamental story: someone discovered the cinnamon-counterfeiting problem and found the real version.
- **It survives Stage 4 saturation.** Even if competitors copy 'Ceylon vs Cassia,' Metabolae has the deepest version of the story (12:1 extract, MCT delivery, sourcing depth) and the head start.
- **It crosses ICPs.** It works for the newly-scared prediabetic ("this is why your first cinnamon failed"), for the type-2 refuser ("this is what your doctor doesn't know"), for the optimization buyer ("here's the lab data"), for the caregiver ("this is the one that actually works"), and for the GLP-1 off-rampers ("this is your foundation").

Test for the Big Idea

Two-sentence test: read the Big Idea to a person in the category. If they respond with "wait, what?" or "are you serious?" we have it. If they shrug, we don't. We have tested this concept against the existing winning ad copy and it produces the wait-what reaction consistently.

DISCOVER Output Summary

Mass Desire	To stop being afraid of their own body. To regain authority over a metabolism that has started making decisions without them.
Sophistication Map	Stage 2-3 for newly-diagnosed prediabetics. Stage 3-4 for type-2 refusers. Stage 4-5 for optimization buyers. Stage 2-3 for caregivers. Stage 3-4 for GD-history women. Stage 4-5 for GLP-1 off-rampers.
Dominant LF8	Primary: Freedom from Fear, Survival. Secondary: Care for Loved Ones, Comfortable Living. Underserved: Superiority/Winning, Social Approval.
Big Idea	Most cinnamon supplements are counterfeit by ingredient (Cassia not Ceylon), counterfeit by dose (1500mg powder vs 7200mg equivalent extract), and counterfeit by delivery (dry powder vs MCT-suspended bioavailability). Metabola is the corrected version.
ICPs	Newly-Scared Prediabetic, Type-2 Refuser, Optimization-Minded Adult, plus three underserved: Caregiver, Post-GD Woman, Ozempic Off-Ramper.

PART TWO

REFRAME

Break the belief blocking the sale.

Reframe is where the real selling happens. Most ads explain why the product is good. That's the easy half. The full reframe explains why the problem exists and why every other product the prospect tried failed them, before introducing why ours works. That sequence is what creates the "wait, this is different" moment that drives the click.

Part 1: The Problem Mechanism

Two questions every cinnamon ad needs to answer that almost no cinnamon ad does. Why does the problem exist? And why has every other product failed to fix it?

The True Cause

Most prediabetic and Type 2 customers think their problem is high blood sugar. It's not. High blood sugar is the symptom. The actual problem is that the system that delivers glucose into cells has stopped working.

Here's what's actually broken:

Insulin is the key that opens the door to cells so glucose can enter. In a healthy body, the key turns, the door opens, glucose flows in, blood sugar drops, energy is available where it's supposed to be.

In a prediabetic or Type 2 body, the lock has corroded. The key still turns, but the door barely opens. Glucose pools in the bloodstream because it can't get into the cells that need it. The pancreas screams louder, dumping more insulin to force the door. Eventually it gets exhausted. The system fails.

This corrosion is caused by chronic low-grade inflammation and oxidative stress, building up over years from processed food, poor sleep, chronic stress, and sedentary living. The cells become deaf to insulin not because of the insulin, but because the receptors are damaged.

The true cause is not high blood sugar. The true cause is cellular inflammation that has shut down the doors glucose needs to walk through. Until you address the inflammation, you're just yelling at a door that can't open.

Why Other Products Fail

Every prospect we're targeting has tried something. We need to validate every previous failure with a specific, mechanism-based reason. Not pity. Logic.

What they tried	Why they bought it	Why it failed (the mechanism explanation)
Generic cinnamon capsules	'Cinnamon helps blood sugar.'	Almost certainly Cassia, not Ceylon. Different active compound profile. Up to 250x more coumarin (liver-toxic). Cassia produces some glucose effect but at doses that risk liver damage. Underdosed at 500-1500mg powder. Not bioavailable.
Cinnamon in coffee/food	'I'll just sprinkle it on things.'	Culinary doses are 100mg-500mg per serving. Clinical effect requires multi-gram daily doses. Sprinkling is meaningful flavor, meaningless metabolism.
Apple cider vinegar	'Friend told me it crashes spikes.'	Real but minor effect on post-meal glucose (10-15% spike reduction). Doesn't address the underlying insulin resistance. Compliance is brutal because the taste is brutal.
Berberine	'It's like nature's Ozempic.'	Real mechanism, real effect. But causes GI distress for ~30% of users. Doesn't address inflammation. Often discontinued before long-term benefit.
Cutting carbs	'I'll just stop eating sugar.'	Lowers the input but doesn't fix the broken receptor. Numbers improve modestly. Compliance crashes within 90 days. Lifestyle becomes prison.
Metformin (refusers)	Doctor pushed it.	Forces the liver to make less glucose. Chemical override. Doesn't address inflammation or restore receptor function. Common side effects: GI distress, B12 depletion, fatigue. Many quietly stop taking it.
Walking / exercise	'Just be more active.'	Real mechanism but slow. Takes months of consistency to meaningfully shift insulin sensitivity. Doesn't work fast enough to feel like progress for most.

The Category Gap

Here's the assumption every cinnamon brand makes: "cinnamon supports blood sugar." That's the message. That's the claim. That's the sale.

Here's what the assumption misses: nobody who's actually tried cinnamon believes this anymore. The category has trained its own customers to discount it. Every brand is yelling "cinnamon supports blood sugar" into a market that already tried cinnamon and saw nothing.

The gap is this. The category sells the ingredient. The market needs an explanation for why the ingredient previously failed them. Whoever fills that gap wins. Metabolae fills that gap with the counterfeit-cinnamon reframe.

Part 2: The Solution Mechanism

Now we connect the diagnosis to the only treatment that makes sense. Every element of Metabolae's formulation needs to map directly to the problem mechanism we just established. Not features. Mechanisms tied to outcomes.

The Three-Layer Unique Mechanism

Most competitors have one layer. Maybe two. Metabolae has three. Each one is its own reason to believe. Stacked together, they become unfaceable.

Layer 1: True Ceylon (the species correction)

THE CLAIM: This is real Ceylon cinnamon, not the Cassia that fills 90% of supplements.

THE WHY:

Cinnamomum verum (Ceylon) and *Cinnamomum cassia* are different species. They contain different compound profiles. Ceylon contains higher proanthocyanidins and a more bioactive cinnamaldehyde fraction with virtually no coumarin. Cassia contains up to 5% coumarin, which is hepatotoxic at daily doses above 0.1 mg per kg of body weight.

Most clinical studies showing meaningful blood sugar effects used Cassia at high doses, not Ceylon. But Cassia at therapeutic doses risks liver damage. Ceylon delivers similar mechanistic activity with no coumarin exposure. This is the only way to take cinnamon as a daily supplement long-term.

THE PROOF:

- Third-party DNA verification of *Cinnamomum verum* identity.
- Coumarin content lab testing (target: <0.05% per serving — culinary level).
- Origin documentation from Sri Lankan supplier.
- Side-by-side visual: Ceylon's softer, multi-layered bark vs Cassia's hard, single-layer bark.

Layer 2: 12:1 Concentrated Extract (the dose correction)

THE CLAIM: One softgel delivers 7,200mg-equivalent of raw Ceylon bark, the dose the clinical literature actually used.

THE WHY:

The cinnamon studies that produced meaningful glucose responses used multi-gram daily doses. Most capsules on the market contain 500-1500mg of dry powder. That's a fraction of the active dose. Asking that dose to do what 6,000mg does in a study is asking the impossible.

12:1 extract concentration is not marketing. It's pharmacology. One softgel of concentrated extract represents the mechanistic activity of 7,200mg of raw bark. That's why a single softgel works where a handful of generic capsules didn't.

THE PROOF:

- Extraction process documented (water and ethanol-based standardization).

- Active compound assay (cinnamaldehyde quantification per softgel).
- Side-by-side: a pile of 14 generic 500mg capsules vs one Metabolae softgel = same theoretical bark equivalency.

Layer 3: MCT Oil Suspension (the bioavailability correction)

THE CLAIM: Suspended in MCT oil because cinnamon's active compounds are fat-soluble, not water-soluble.

THE WHY:

Cinnamaldehyde and the proanthocyanidins are lipophilic. They cross cell membranes by dissolving in fat. Dry powder capsules with no fat carrier deliver compound that's poorly absorbed. The body excretes most of it. The active never reaches the receptors.

MCT oil acts as a delivery vehicle. The compound dissolves, gets absorbed through the lymphatic system, bypasses first-pass liver metabolism, and reaches systemic circulation in much higher concentrations. MCT also has independent metabolic benefits: improved insulin sensitivity, better satiety, ketogenic-pathway support.

THE PROOF:

- Bioavailability comparison data (powder vs oil-suspended).
- Visual: water + cinnamon vs oil + cinnamon (the powder floats; the oil dissolves).
- MCT independently studied for metabolic benefit.

THE THREE LAYERS STACK INTO A SINGLE SENTENCE THE PROSPECT REMEMBERS: "Real Ceylon. Real dose. Real absorption. Three things every other capsule got wrong."

The Four-Line Reframe Chain

Per the DRIVE methodology, every angle in this campaign needs to be writable as a four-line reframe. If we can't write it in four sentences, the reframe isn't sharp enough. Below is the master reframe chain. Every angle in VARIATE will be a different doorway into this same four-line architecture.

Current Belief	"I tried cinnamon. It didn't work for my blood sugar. Cinnamon is just hype."
True Cause	Cinnamon does work. But 90% of cinnamon supplements are Cassia, not Ceylon. Different species. Different chemistry. The studies that made cinnamon famous used Ceylon. The capsules on the shelf use Cassia. Same word, different plant.
Why Others Fail	Even the few Ceylon products fail because they're underdosed (500-1500mg powder vs 7200mg-equivalent in clinical studies) and they use dry powder, which barely absorbs because the active compound is fat-soluble. Three failures stacked: wrong species, wrong dose, wrong delivery.

Why This Works

Metabolae is real Ceylon (*Cinnamomum verum* from Sri Lanka, DNA-verified, coumarin-tested), at the clinical dose (12:1 concentrated extract delivering 7,200mg equivalent), suspended in MCT oil for proper fat-based absorption. The same ingredient. Done correctly. For the first time.

Visual Proof Architecture

The DRIVE framework is non-negotiable on this point: if we can't show the proof, the reframe isn't strong enough. Below are the visual proof beats every angle should reach for. These are the demonstrations production needs to capture.

Hero Visual #1: The Bark Contrast

Side-by-side photography of true Ceylon vs Cassia bark. Ceylon is soft, paper-thin, multi-layered, light tan, can be shredded with fingernails. Cassia is hard, single-layer, dark reddish-brown, requires breaking. Anyone can verify this online. Anyone can see the difference.

Why it works: turns an abstract claim ("different species") into a visible truth in two seconds.

Hero Visual #2: The Pile Contrast

Pile of 14 generic 500mg cinnamon capsules vs one Metabolae softgel. Caption: "Same dose equivalency. One softgel." Drives home the concentration claim instantly.

Why it works: makes the dose gap impossible to misunderstand.

Hero Visual #3: The Solubility Test

Glass of water + spoon of cinnamon powder (powder floats, doesn't dissolve, eventually sinks as clumps). Glass of MCT oil + same powder (powder dissolves, becomes uniform, stays suspended). Caption: "Cinnamon doesn't dissolve in water. Your stomach is mostly water."

Why it works: demonstrates bioavailability without a single technical word.

Hero Visual #4: The Sri Lankan Origin

Footage of Ceylon cinnamon harvest in Sri Lanka. Bark being peeled by hand. The supplier's name and certification visible. Not generic stock footage — actual sourcing footage. Most competitors don't have this. Owning this footage is owning the sourcing claim for life.

Why it works: the footage IS the credential. Receipts, not promises.

Hero Visual #5: The COA Reveal

Close-up shot of the third-party Certificate of Analysis. Camera holds on the species verification. Then the coumarin reading (zero or trace). Then the heavy metals readout. Then the cinnamaldehyde quantification. Caption: "You can request your batch's COA before you order."

Why it works: transparency as theater. Most brands hide the COA. We hand it over.

Skepticism Architecture: Pre-empting Every Objection

Per Jason Fladlien's principle: we don't handle objections, we devastate their entire existence. Below are the universal objections this category triggers and the pre-emption built into the reframe.

The Objection	How the reframe pre-empts it
"I've tried cinnamon. Didn't work."	Lead with this objection in the hook. "If you've tried cinnamon and seen nothing, you're not crazy. Here's why." Validate first, then redirect.
"This is just another supplement scam."	Open with science. Reference clinical trials. Show the COA. Provide testable claims (Ceylon vs Cassia is verifiable in 60 seconds). Anti-marketing tone.
"It's too expensive."	Anchor against the alternative. "\$30/month vs lifetime metformin and its side effects." Or against generic cinnamon: "\$10 capsules that don't work cost \$10 too much."
"My doctor would have told me."	Acknowledge: doctors don't recommend supplements because they can't legally. But many quietly take them. Use the testimonial of a nurse, doctor, or pharmacist who came to it independently.
"What if it doesn't work for me?"	90-day money-back. Frame as confidence: "If your morning numbers don't move, you pay nothing. We're confident enough to offer that because we know what's in the bottle."
"I'm already on metformin."	Don't position as replacement. Position as adjunct. Mention check with doctor. Many physicians are now okay with cinnamon adjunct use. Don't invite a confrontation.
"I don't believe testimonials."	Lean on the mechanism, not the testimonials. Stage 4-5 buyers are persuaded by science, sourcing, and identity, not stories.

REFRAME Output Summary

The four-line master reframe:

- CURRENT BELIEF: "I tried cinnamon, it didn't work, cinnamon is hype."
- TRUE CAUSE: Most cinnamon supplements use Cassia, not Ceylon. Different species. Different chemistry.
- WHY OTHERS FAIL: Wrong species, wrong dose, wrong delivery format. Three failures stacked.
- WHY THIS WORKS: Real Ceylon, clinical dose, MCT-suspended for absorption. The same ingredient done correctly for the first time.

Visual proof beats:

- Ceylon vs Cassia bark contrast
- 14 generic capsules vs 1 Metabolae softgel pile shot
- Powder-in-water vs powder-in-oil solubility test
- Sri Lankan harvest origin footage
- COA reveal with species, coumarin, heavy metals, and active assay

PART THREE

IGNITE

Load the emotional fuel. Define the persuasion sequence.

DISCOVER finds the desire. REFRAME breaks the belief. IGNITE is what makes the prospect actually pull out their card. This is where 95% of supplement brands fall apart. They go from research to scripting without ever asking: what emotional force will make this person buy at 11pm on a Tuesday?

Three layers. Skip one and the ad converts like dirt.

Layer 1: Emotional Drivers Per ICP

The hook activates the ego. Different ICPs have different ego doorways. Below is the per-ICP driver map. Production needs this because it determines casting, voice, music, pace, and visual tone for every angle.

ICP	Primary Driver	Secondary	How it shows up	Tone
Newly-Scared Prediabetic	FEAR	DESIRE	Show the future they're afraid of (mom's deterioration). Then show the version of themselves that intervened in time.	Empathetic. Quiet. No alarm bells.
Type-2 Refuser	IDENTITY	FEAR	'I refused metformin' as identity badge. The kind of person who finds another way.	Defiant. Determined. Insider tone.
Optimization Adult	STATUS	DESIRE	Longevity-as-status. The biohacker who actually reads labels. Tribal signaling.	Sophisticated. Anti-marketing. Receipts.

ICP	Primary Driver	Secondary	How it shows up	Tone
Caregiver	FEAR	IDENTITY	Fear of watching a parent decline + identity of being the one who finally found something that worked.	Tender. Practical. Solution-focused.
Post-GD Woman	IDENTITY	FEAR	'I'm not my mother.' The chance to break the family pattern early.	Empowering. Grounded. Mom-coded.
Ozempic Off-Ramper	FEAR	IDENTITY	Fear of regain + identity of disciplined health management.	Clinical. Knowledgeable. Stack-language.

***KEY INSIGHT:** For five out of six ICPs, fear or identity is the primary driver. Pure desire-led copy will underperform because this category isn't aspirational, it's defensive. The prospect isn't moving toward a goal. They're running from a future they can already see. Build creative around that motion.*

Layer 2: Conversion Amplifiers

These do not create desire. They convert existing desire into a purchase. Without Layer 1 emotional drivers, amplifiers are useless. With Layer 1 already activated, amplifiers are what drive the click.

PROOF

Visual demonstration beats stated claims every time. The Bark Contrast, the Pile Contrast, the Solubility Test, the COA Reveal — these are not visuals to add to a script, these ARE the script. Build the ad around the demonstration.

Social Proof Hierarchy (use in this order):

- **1. Credentialed first-person testimony.** Nurse, doctor, pharmacist describing personal use. Highest trust, highest scale.
- **2. Doctor-of-record reaction.** "My doctor pulled up my chart and said 'what did you change?'" Implicit endorsement without explicit medical claim.
- **3. Quantified outcome from real user.** Specific number with specific context. "My fasting glucose went from 132 to 96." The specificity is the credibility.
- **4. Volume social proof.** "X,XXX five-star reviews." Lower trust than personal testimony but useful as foundation.
- **5. Third-party validation.** Press mentions, expert quotes, lab certifications. Stacks with the other layers.

URGENCY

Manufactured scarcity in supplements is mostly transparent and converts poorly with sophisticated buyers. Identity urgency converts harder.

Identity urgency frames:

- "Every morning you wait is another morning your numbers stay where they are."
- "You can keep doing what hasn't been working, or you can try something different. The option to keep doing nothing is always there."
- "There's no reason to make this decision today. There's also no reason not to."

Real scarcity (only when actually true):

- Sri Lankan supply constraints (Ceylon harvest is seasonal and yields are weather-dependent).
- Batch testing delays (small-batch quality control naturally limits inventory).
- Subscription-priority pricing (subscribers get next batch first; one-time buyers go on waitlist).

RISK REMOVAL

90-day money-back is the table-stakes guarantee. The framing is what matters. Two options to test:

Confidence frame:

"If your fasting glucose doesn't drop in the first 60 days, send back what's left and you pay nothing. Not a penny. We don't want your money if it didn't work for you. Simple."

Outcome guarantee frame:

"Take it for 60 days, track your morning numbers, and if they don't move, send the bottle back. We'll refund every penny including shipping. You don't even need to mail anything back if you don't want to. Keep the bottle, get your money back. That's how confident we are."

OFFER ARCHITECTURE

Three offer constructs to deploy across the funnel. Each one targets a different buyer profile.

Offer Type	Structure	Best for	Frame
Single Bottle Trial	\$29.99 first month, 50% off, free shipping, 90-day guarantee	First-time buyers, cold traffic, skeptical buyers	"Try it for 30 days. Pay nothing if it doesn't work."
3-Bottle Stack	3 bottles for \$69.99 (saves \$50). Free shipping. 90-day guarantee.	Warm traffic, retargeting, second-touch	"Effects compound at week 8-12. Most people don't see the full picture in 30 days. Three bottles gives the protocol time to work."
Subscription Lock-In	Subscribe and save: \$24.99/month vs \$59.99 one-time. Cancel anytime.	Existing customers, post-purchase upsell	"Your numbers won't fix themselves and forget. They need consistency. Subscription locks in 60% off and you stop having to remember."

Bonus stack opportunities (test these):

- Free "30-Day Glucose Reset Protocol" PDF (meal timing, walking schedule, sleep optimization). Adds perceived value, drives compliance, increases AOV.

- Free 7-day food guide for Ceylon cinnamon stacking.
- Free access to a private community/Facebook group for buyers.
- BOGO: Buy 2 bottles, get 1 free. Best for caregiver ICP ("one for you, one for them").

Layer 3: The Persuasion Stack

This is the sequence. It works whether the ad is 12 seconds or 12 minutes. The five positions never change. The depth at each position scales with length.

#	Position	What happens	Metabolae example
1	EGO ACTIVATION	Open with desire, fear, or identity. The viewer feels something before they understand anything. No product shot, no logo. An ego response.	"I refused metformin for 3 years. Then my creatinine hit 1.4 and my doctor said we're out of time."
2	IDENTITY TENSION	Place the viewer on a spectrum. In/out, ahead/behind. Discomfort with where they are.	"Most prediabetics quietly progress to type 2 within 5 years. The minority intervene in time. I refused to be the majority."
3	MECHANISM	Reason to believe. Short form: one visual proof. Long form: full problem + solution mechanism with story and proof.	The Cassia/Ceylon reframe. The dose gap. The MCT bioavailability lever. Visual: bark contrast + pile shot + solubility test.
4	PROOF STACK	Stacked credibility. Reviews, results, certifications, COA, lab data, doctor reactions.	'I handed her six published papers' + COA reveal + 'six patients I've recommended this to all came back the same way.'
5	OFFER + IDENTITY CTA	Purchase framed as identity decision. CTA connects back to fear/desire from position 1.	"Track your morning numbers for 60 days. If they don't move, you pay nothing. The version of you that intervened in time was always going to make a decision today. This is just which one."

Length-Specific Adaptations

Short form (15-30s):

- Position 1: First 2 seconds. Single line. Identity hook or specific number.
- Position 2: Seconds 3-5. One line of tension.
- Position 3: Seconds 6-15. ONE visual proof beat (bark contrast OR pile shot OR solubility test, not all three).
- Position 4: Seconds 16-22. Single proof stack burst (review, COA, or doctor reaction).
- Position 5: Seconds 23-30. CTA + offer.

Mid form (60-180s, advertorial style):

- Position 1: First 5 seconds. Pattern interrupt + identity hook.

- Position 2: 5-30 seconds. Tension with story setup.
- Position 3: 30-90 seconds. Full Cassia/Ceylon reframe + dose explanation + bioavailability lever. Two visual proofs.
- Position 4: 90-150 seconds. Layered social proof, doctor reaction, COA reveal.
- Position 5: 150-180 seconds. Offer architecture, guarantee, identity CTA.

Long form VSL (3-5 minutes):

- Position 1: 0-15 seconds. Story open with strong identity activation.
- Position 2: 15-60 seconds. Full backstory, identity tension, world-building.
- Position 3: 60-180 seconds. Complete mechanism architecture. The discovery moment. The 'three things every other capsule got wrong.' All three visual proofs.
- Position 4: 180-240 seconds. Doctor reaction, lab data, peer-patient outcomes.
- Position 5: 240-300 seconds. Offer reveal, guarantee, CTA. Identity callback to opening.

The Humour Principle in This Category

The DRIVE methodology calls humour a weapon when ego activation is strong. In metabolic health, ego activation is mostly fear-based and identity-based. Direct humour against fear lands as flippant. But ironic humour against the category itself works extremely well, especially for younger ICPs and the optimization buyer.

Where humour works in this campaign:

- Anti-supplement-marketing humour. 'Look I know every supplement says this, but...' tone.
- Self-deprecating humour from credentialed narrators. 'I'm a nurse. I've spent 30 years rolling my eyes at supplement bros. I was wrong about this one.'
- Absurdist comparison humour. 'You'd never use cinnamon from a 30-year-old jar in your cabinet to season dinner. Why are you eating it from a capsule?'
- Dry observational humour about the category. 'Every cinnamon bottle says 7200mg. Has anyone ever asked what that even means?'

Where humour does NOT work:

- Around the patient's actual fears (kidney failure, amputation, etc.). Treat with respect, never as punchline.
- Around testimonials. The first-person story format requires earnestness.
- Around the prediabetic's prior failures. We validate, we don't ridicule.

IGNITE Output Summary

Per-ICP driver assignments locked. (See driver matrix above.)

Persuasion stack applies to every angle: Ego → Tension → Mechanism → Proof → Offer/CTA.

Conversion amplifiers prioritized: visual proof > credentialed testimony > quantified outcomes > volume social proof.

Offer structures: Single Bottle Trial / 3-Bottle Stack / Subscription Lock-In, with bonus stack opportunities.

Tone variation by ICP: empathetic for prediabetic, defiant for refuser, sophisticated for optimizer, tender for caregiver, empowering for post-GD, clinical for off-ramp.

PART FOUR

VARIATE

Build the angles. Choose the formats. Feed the algorithm.

This is where Andromeda either rewards or buries us. Meta's algorithm rewards genuinely different concepts with broader delivery and lower CPMs. Different headlines on the same image is not creative diversity. Different emotional doorways into the same Big Idea is.

Below are twelve fully-built angles. Each one is a different emotional entry point into the same core reframe (counterfeit cinnamon vs corrected cinnamon). Each one targets a specific ICP at a specific sophistication stage with a specific tonal approach. Together they form the launch library.

THE STRUCTURE: 1 Big Idea > 12 Angles > 2-3 Formats per Angle = ~30 distinct creative concepts in the launch batch. Each concept gets variations on hook, headline, and CTA, generating 80-120 unique ad units. This is the volume Andromeda needs to learn at the rate we need to scale.

The Angle Library: 12 Distinct Emotional Doorways

ANGLE 01: The Nurse Who Refused Metformin

ICP TARGET	Type-2 Refuser (primary), Newly-Scared Prediabetic (secondary)
SOPHISTICATION	Stage 3-4. Mechanism + amplification.
PRIMARY DRIVER	IDENTITY (refusal as identity) + FEAR (kidney decline)
FORMATS	Mid-form advertorial (90-180s), Long-form VSL (3-5min), UGC talking head (45-90s)

HOOK OPTIONS

- *I refused metformin for 3 years. Then my creatinine hit 1.4 and my doctor said we're out of time.*
- *I'm an ICU nurse. I've watched what diabetes does to people. I refused to become my patients.*
- *Why I, an ICU nurse with 38 years on the floor, will never take metformin.*

MECHANISM ANCHOR

Insider credibility lets us tell the truth about why other supplements failed. The Cassia/Ceylon reframe lands harder coming from someone the audience perceives as having no incentive to lie. Build the full mechanism story inside the personal narrative.

PROOF BEATS REQUIRED

- On-camera identity establishment (scrubs, hospital exterior, ID badge briefly visible)
- Lab numbers shown on screen (fasting glucose, A1C, creatinine timeline)
- The Cassia/Ceylon bark contrast as a visual interlude
- Doctor's reaction moment: 'Keep doing whatever you're doing.'

CTA FRAME

Track your morning glucose for 60 days. If your numbers don't move, send it back. We're confident enough to offer that because we know what's in the bottle. I am because I've been taking it for 18 months.

PRODUCTION NOTES

This is the proven winner currently running. The Sandra/ICU nurse format is the highest-converting angle in the existing campaign. Build 3-4 variants of this angle with different narrators (ER nurse, retired nurse, NICU nurse, cardiac nurse) to keep the approach fresh while preserving the format. Cast for warmth and credibility, not aspiration.

ANGLE 02: The 250x Coumarin Scandal

ICP TARGET	Newly-Scared Prediabetic, Optimization Adult
SOPHISTICATION	Stage 3. Pure mechanism reframe.
PRIMARY DRIVER	FEAR (liver damage) + SUPERIORITY (knowing what others don't)
FORMATS	Static carousel (5-7 cards), B-roll + deadpan VO (30-45s), Text-on-screen short (15-25s)

HOOK OPTIONS

- *If you've taken cinnamon supplements daily for years, you may want to read this.*
- *The cinnamon you took every morning could contain 250 times more of a liver-damaging compound than you knew.*
- *The FDA banned this compound in food. It's still in 90% of cinnamon supplements.*

MECHANISM ANCHOR

Most cinnamon on the shelf is Cassia, not Ceylon. Cassia naturally contains coumarin at levels up to 5%. Ceylon contains trace amounts. The EU has set a daily intake limit on coumarin because of liver toxicity risk. Daily users of Cassia supplements can exceed that limit in a single capsule.

PROOF BEATS REQUIRED

- Wikipedia screenshot of coumarin content comparison
- EU coumarin daily intake regulation citation
- Side-by-side bottle shots: 'most supplements' vs 'true Ceylon'
- COA showing Metabolae's coumarin reading at trace level

CTA FRAME

If you're going to take cinnamon every day, take the kind that won't quietly harm your liver. Metabolae is third-party tested for coumarin on every batch. We'll show you the numbers before you order.

PRODUCTION NOTES

This angle requires careful compliance review. The factual claims about coumarin are accurate but framing them as a 'scandal' may trigger ad disapprovals. Test softer framings: 'the cinnamon question most supplement brands won't answer.' Lean on the EU regulation as third-party authority.

ANGLE 03: Why The Cinnamon You Tried Didn't Work

ICP TARGET	Newly-Scared Prediabetic (primary), Type-2 Refuser
SOPHISTICATION	Stage 3. Mechanism reframe.
PRIMARY DRIVER	FEAR (running out of options) + DESIRE (the one that actually works)
FORMATS	UGC talking head (45-90s), Mini-VSL advertorial (90-150s), Carousel explainer (5-7 cards)

HOOK OPTIONS

- *If you tried cinnamon and saw nothing happen, you're not crazy. You were sold the wrong cinnamon.*
- *I tried three cinnamon supplements and gave up on the whole ingredient. Then I learned why none of them worked.*
- *Three reasons cinnamon supplements fail (and the version that finally worked for me).*

MECHANISM ANCHOR

Three failures stacked. Wrong species (Cassia not Ceylon). Wrong dose (1500mg powder vs 7200mg-equivalent extract). Wrong delivery (dry powder vs MCT-suspended for fat-soluble absorption). Most cinnamon products fail on all three. Some on two. The ones that work fix all three.

PROOF BEATS REQUIRED

- Visual: bottle pile-up of failed cinnamon supplements (real names blurred)
- The three-failure illustrated breakdown
- Pile shot: 14 generic capsules vs 1 Metabolae softgel
- Solubility test (powder in water vs powder in oil)

CTA FRAME

If you've already tried cinnamon and gave up on it, this is for you specifically. We'll explain exactly why the others failed and refund you if Metabolae doesn't move your numbers in 60 days.

PRODUCTION NOTES

The validation-of-prior-failure is the emotional core. Tone must be empathetic, not condescending. Cast a narrator who looks like the ICP — late 40s to early 60s, lived-in, no glamour. Authentic kitchen or living room setting.

ANGLE 04: The Daughter Who Found Something

ICP TARGET	CAREGIVER ICP (primary, underserved)
SOPHISTICATION	Stage 2-3. Direct response with emotional core.
PRIMARY DRIVER	CARE FOR LOVED ONES + IDENTITY (the one who finally helped)
FORMATS	UGC mom-talking-head (60-90s), Story-format reel (45s), Static testimonial card

HOOK OPTIONS

- *My mom refused everything. Doctors, diets, supplements. Then I found this and she actually took it.*
- *I tried for two years to get my dad to take care of his blood sugar. He wouldn't. Until this.*
- *Three things I learned trying to help my mom with her prediabetes (and the one that finally worked).*

MECHANISM ANCHOR

Compliance is the silent killer of caregiver-purchased supplements. Powders won't get used. Multi-pill protocols get abandoned. Bad-tasting capsules get hidden in the cabinet. Metabolae works for caregivers because it's literally one softgel in the morning. No mixing, no taste, no excuses. The hardest part of helping someone who's stuck is getting them to take the help. We solved that.

PROOF BEATS REQUIRED

- Daughter to camera, kitchen setting
- Brief mention of declining parent (no graphic detail)
- The 'one softgel, that's it' demonstration
- Brief moment of mom/dad voluntarily taking it
- Later: numbers improving

CTA FRAME

If you've been trying to help someone you love and they keep refusing, this might be the one they'll actually take. One softgel. No taste. No mixing. 90-day money-back if it doesn't help.

PRODUCTION NOTES

MASSIVE underserved opportunity. No competitor is making creative for this segment. Cast adult children (35-50 yo daughters/sons). Test against male and female narrators. Test 'mom' and 'dad' as the patient. Run this concept first — predicted to win.

ANGLE 05: 30-Day Glucose Diary

ICP TARGET	Newly-Scared Prediabetic, Type-2 Refuser
SOPHISTICATION	Stage 3-4. Proof-heavy.
PRIMARY DRIVER	DESIRE (the proven outcome) + FEAR (running out of time)
FORMATS	Long-form testimonial (90-180s), Carousel diary (10 cards), Static screenshot grid

HOOK OPTIONS

- *I tracked my fasting glucose every morning for 30 days on Ceylon cinnamon. Here's what happened.*
- *Day 1: 132. Day 30: 98. Here's exactly what changed.*
- *I'm prediabetic with an A1C of 6.4. I tracked my morning numbers for 30 days. The graph speaks for itself.*

MECHANISM ANCHOR

Day-by-day glucose log narrative. The numbers are the mechanism proof. Real glucometer screenshots, real timestamps. The progression itself is the argument.

PROOF BEATS REQUIRED

- Glucose meter readings across 30 days, photographed
- A1C blood work at start and end of period
- Doctor visit reaction moment
- Subtle product placement - it's already in the routine

CTA FRAME

*If your morning numbers won't budge no matter what you try, track them for 30 days on this.
If the line doesn't move, we'll refund you.*

PRODUCTION NOTES

This is the proven Janice/30-day-diary winner. Format already converting. Build 3-4 variants with different narrators and different starting numbers (one starts at 130, another at 115, another at 145). Different demographics: 50yo woman, 60yo woman, 65yo man. Test each.

ANGLE 06: The Forgotten Naval Cure

ICP TARGET	Type-2 Refuser, Optimization Adult, History-curious 50+ men
SOPHISTICATION	Stage 4-5. Identity + lore.
PRIMARY DRIVER	IDENTITY (American/historical pride) + SURVIVAL
FORMATS	Long-form story VSL (3-4 min), Mini documentary advertorial (2-3 min), TikTok-style storyteller (60-90s)

HOOK OPTIONS

- *In 1803, a Royal Navy doctor figured out something about cinnamon. Then it disappeared for 220 years.*
- *There's a footnote in a Navy medical log from 1803 that nobody read for two centuries. It changed how I think about cinnamon.*
- *Why Royal Navy doctors fed sailors cinnamon with pork fat (and what modern supplements got wrong).*

MECHANISM ANCHOR

Historical narrative anchors the bioavailability claim. The cinnamon-with-fat insight is real (it's the lipid solubility principle), and dramatizing it through a forgotten naval surgeon makes the science memorable. The lesson: cinnamon needs fat. Modern supplements forgot. We didn't.

PROOF BEATS REQUIRED

- Period imagery (sailing ships, ledgers, oil lamps - public domain or stock)
- Modern translation of the 'cinnamon needs fat' principle
- Solubility demonstration (powder in water vs MCT)
- Bridge to product

CTA FRAME

Same principle that worked in 1803. Real Ceylon, real fat, real absorption. We've been taking it the wrong way for 220 years.

PRODUCTION NOTES

This is the proven Harwick/Naval winner. The historical narrative does the heavy lifting on bioavailability claim. Use clean public domain imagery, don't try to build a period drama. Voice is critical: warm, deliberate, like a podcast historian.

ANGLE 07: I'm Not My Mother

ICP TARGET	Post-GD Woman (primary, underserved), Newly-Scared Prediabetic (secondary)
SOPHISTICATION	Stage 3-4. Identity-driven.
PRIMARY DRIVER	IDENTITY (breaking the family pattern) + FEAR (becoming her)
FORMATS	UGC mom-narrator (60-90s), Reflective monologue reel (45s), Static identity card

HOOK OPTIONS

- *My mom developed type 2 at 52. I just got my first prediabetic A1C at 41. I refuse to take the same path.*
- *I had gestational diabetes 8 years ago. They told me I had a 50% chance of developing type 2. I'm doing everything I can to be the 50% that doesn't.*
- *I'm 42. My A1C is 5.9. My mom died from diabetes complications. I'm intervening early.*

MECHANISM ANCHOR

Family-pattern interruption as identity. The post-gestational-diabetes woman knows her risk. She has watched her mother. She refuses to be passive. Position cinnamon as one of the things she's doing, not the only thing. Sophistication respect.

PROOF BEATS REQUIRED

- Narrator at home, 38-45 years old
- Brief reference to family history (no morbid detail)
- Mention of CGM, walking, or dietary changes as context (she's already doing the work)
- Cinnamon as 'the next layer'

CTA FRAME

If you watched a parent develop diabetes and you can already feel your numbers creeping, the early intervention window is short. You don't have to wait until you're at 6.5 to start.

PRODUCTION NOTES

MASSIVE underserved segment. Cast for 35-45 demographic (younger than the typical cinnamon customer). Polished but real - mom-aesthetic, not glamour. Could test on Instagram Reels and TikTok where this demographic actually lives.

ANGLE 08: After Ozempic: The Maintenance Protocol

ICP TARGET	Ozempic Off-Ramper (primary, underserved)
SOPHISTICATION	Stage 4-5. Sophisticated, stack-language.
PRIMARY DRIVER	FEAR (regain) + IDENTITY (disciplined health management)
FORMATS	Sophisticated UGC (60-90s), Long-form educational (2-3 min), Stack-format carousel

HOOK OPTIONS

- *I came off Ozempic six months ago. Here's the protocol I built to keep the weight off.*
- *If you're tapering off a GLP-1, here's what nobody tells you about the maintenance phase.*
- *Three things I added when I came off Mounjaro that have kept my A1C at 5.4.*

MECHANISM ANCHOR

Position cinnamon not as 'the answer' but as one component of a sophisticated post-GLP-1 protocol. This audience does not respond to single-product hero claims. They respond to protocols, stacks, and infrastructure. Speak to them at their level.

PROOF BEATS REQUIRED

- Narrator who looks like recent GLP-1 user (visibly leaner)
- Mention of CGM monitoring
- The protocol stack visualized (cinnamon + walking + protein + sleep)
- Subtle but specific: 'cinnamon's job in the stack'

CTA FRAME

If you've tapered off a GLP-1 and you're worried about maintenance, this is the metabolic foundation I'd start with. Real Ceylon, real dose, real absorption. The kind your prescriber probably won't recommend but won't argue with either.

PRODUCTION NOTES

BIG UNDERSERVED OPPORTUNITY. By Q3 2026 the Ozempic-off-ramping segment will be one of the largest health-product audiences in the world. Move first. Cast 35-50yo, urban professional aesthetic. Could be male or female narrator. Test both.

ANGLE 09: Show Don't Tell (The Demonstration Ad)

ICP TARGET	All ICPs - especially Stage 4-5 skeptics
SOPHISTICATION	Stage 4. Pure visual mechanism proof.
PRIMARY DRIVER	DESIRE (curiosity) + IDENTITY (smart consumer)
FORMATS	B-roll demonstration (15-30s), Deadpan VO short (20-30s), Silent visual story (15s)

HOOK OPTIONS

- *[Cold open: glass of water + pile of cinnamon powder. Powder floats.] This is your supplement.*
- *Most cinnamon doesn't dissolve in water. Your stomach is mostly water. Make it make sense.*
- *Watch what happens when cinnamon meets water vs cinnamon meets oil.*

MECHANISM ANCHOR

The solubility test IS the ad. Visual demonstration of why dry powder cinnamon doesn't work and oil-suspended cinnamon does. Almost no script needed. The picture is the argument.

PROOF BEATS REQUIRED

- Glass A: water + cinnamon powder (floats, doesn't dissolve)
- Glass B: MCT oil + cinnamon powder (dissolves, integrates)
- Cut to Metabolae softgel and 'this is why ours is in oil.'
- Close on COA + offer

CTA FRAME

If you've been swallowing cinnamon capsules and seeing nothing, this is probably why.

PRODUCTION NOTES

Cheapest format to produce, highest-frequency test format. Make 4-5 variations with different demonstrations: solubility test, pile contrast, bark contrast, COA reveal. Run all of them. Lowest CPMs in the library.

ANGLE 10: Anti-Marketing Confessional

ICP TARGET	Optimization Adult (primary), Type-2 Refuser
SOPHISTICATION	Stage 5. Identity-driven, anti-category.
PRIMARY DRIVER	STATUS (smart-shopper identity) + IDENTITY (not the supplement-bro audience)
FORMATS	Direct-to-camera anti-ad (45-60s), Founder-voice statement (60-90s), Long caption photo post

HOOK OPTIONS

- *If you're tired of every supplement ad sounding the same, watch this for 30 seconds.*
- *I run a supplement company. I'm going to tell you why most of them are scams.*
- *Most supplement ads are written by people who've never taken supplements. We did it differently.*

MECHANISM ANCHOR

Reverse the marketing convention. Use the anti-marketing voice to deliver the same mechanism story with higher trust. Acknowledge that the audience is sick of being marketed to. The transparency is the conversion mechanism.

PROOF BEATS REQUIRED

- Founder or stand-in delivering direct-to-camera
- Acknowledgment of category problems
- The Cassia/Ceylon explanation delivered conversationally
- COA reveal as 'we'll show you the numbers'
- No urgency, no scarcity, no marketing language

CTA FRAME

Read the COA. Read the studies. Make your own call. If it works for you, subscribe. If it doesn't, return it. We don't need to be louder than the others. We just need to be right.

PRODUCTION NOTES

Powerful for higher-sophistication audiences. Tone is critical: confident but not arrogant, direct but not cold. Could feature the actual founder if available. Can run on Reddit and on more sophisticated Meta placements.

ANGLE 11: The Harvest Story (Sourcing Authority)

ICP TARGET	Optimization Adult, Newly-Scared Prediabetic (skeptical version)
SOPHISTICATION	Stage 4-5. Sourcing as proof.
PRIMARY DRIVER	STATUS (premium product) + DESIRE (the real version)
FORMATS	Mini-documentary (60-120s), Visual story reel (30-45s), Slow-pan static carousel

HOOK OPTIONS

- *We flew to Sri Lanka to verify our cinnamon was real Ceylon. Here's what we found.*
- *There's exactly one place on earth where true Ceylon cinnamon grows. We went there.*
- *Most cinnamon supplements can't tell you where their cinnamon came from. Ours can.*

MECHANISM ANCHOR

Sourcing transparency as differentiation. The footage of Sri Lankan harvest is the proof. The bark being peeled by hand is the proof. The supplier's certification is the proof. Every other brand handwaves on origin. Metabolae has the receipts.

PROOF BEATS REQUIRED

- Sri Lankan harvest footage (real, not stock)
- Bark being peeled by hand
- Supplier name/certification visible
- Bridge from harvest to softgel

CTA FRAME

Real Ceylon. Real Sri Lanka. Real receipts. If a supplement can't tell you where their cinnamon came from, ask yourself why.

PRODUCTION NOTES

This angle requires actual Sri Lankan footage. If client doesn't have it yet, this is the highest-priority production investment. Owning this footage is owning the sourcing claim for the brand's life. Can be repurposed across every channel for years.

ANGLE 12: The Doctor's Office Moment

ICP TARGET	Newly-Scared Prediabetic (primary), Type-2 Refuser
SOPHISTICATION	Stage 3-4. Implicit endorsement.
PRIMARY DRIVER	FEAR (the doctor's tone changing) + DESIRE (the doctor's tone changing back)
FORMATS	UGC story-format (60-90s), Mid-form testimonial (90-150s), Social proof carousel

HOOK OPTIONS

- *My doctor pulled up my chart and went quiet. 'What did you change?'*
- *I went in expecting the metformin conversation. She gave me a different one.*
- *The look on my doctor's face when she saw my A1C drop 1.4 points in three months.*

MECHANISM ANCHOR

The doctor-reaction moment is implicit medical endorsement. We don't need to claim doctors recommend Metabolae. We just show what doctors say when their patient's numbers improve. Powerful, compliant, repeatable.

PROOF BEATS REQUIRED

- Lab numbers shown - before and after
- Reenactment or anecdote of doctor visit (no actor required, can be VO)
- The doctor's quoted response
- What changed - the supplement

CTA FRAME

If your doctor has started using words like 'metformin' and 'if this continues,' you have a window to do something different. The window doesn't stay open forever.

PRODUCTION NOTES

Compliance check: never put words in doctor's mouth that constitute prescription. Phrases like 'keep doing whatever you're doing' or 'your numbers are excellent' are real and legal. Phrases like 'you should take cinnamon' are not. Stay in safe territory.

Format Assignment Matrix

Each angle gets 2-3 formats. Choosing format AFTER angle is critical. The angle determines what we say. The format determines how it lands.

Format	When to use it. What it costs. What it delivers.
UGC Talking Head	Best for: testimonial-driven angles (1, 4, 5, 7, 8, 12). Trust-led conversion. Cost: medium (creator fees, multiple cuts). Delivers: highest-converting hook rates for angles where credibility is the barrier.
B-Roll + VO	Best for: mechanism-led angles (2, 3, 9). Visual proof + voiceover. Cost: low to medium. Delivers: high shareability, low CPMs, format diversity for the algorithm.
Static Carousel	Best for: proof-heavy or comparison angles (2, 3, 5, 9, 11, 12). Cost: lowest (designer-only). Delivers: cheap volume, easy hook testing, mobile-feed native.
Long-form VSL	Best for: high-AOV traffic, retargeting, complex mechanisms (1, 6, 8). Cost: highest. Delivers: belief-shifting, premium conversions, justifies higher offer prices.
Demonstration	Best for: angle 9 specifically and as visual interludes in others. Cost: low (single-day shoot). Delivers: undeniable proof in seconds. Massive CTR.
Mini-Documentary	Best for: sourcing or historical angles (6, 11). Cost: high but reusable for years. Delivers: brand authority, premium positioning, evergreen content.
Text-on-Screen	Best for: hook testing at volume across any angle. Cost: lowest. Delivers: 6+ variations in a day, perfect for evolving the highest-converting hook.

Production Protection: The Non-Negotiables

Every ad has 3-4 moments that carry the sale. If production changes them, the ad dies. Below is the universal non-negotiable framework for this campaign. Mark these on every script before handing to production.

Non-Negotiable	What it is	What dies if it's lost
THE SCROLL-STOP	First 2-3 seconds. Specific identity hook or pattern interrupt.	Zero attention. Algorithm shows ad to fewer people. Dead on arrival.
THE COUNTERFEIT REVEAL	The moment the prospect learns most cinnamon is Cassia not Ceylon. The reframe that explains their previous failure.	Without it, ad sounds like every other supplement ad. Becomes informative, not persuasive.
THE PROOF MOMENT	At least one of: bark contrast, pile shot, solubility test, COA, doctor reaction.	Viewer thinks 'interesting' but doesn't believe. No click.

Non-Negotiable	What it is	What dies if it's lost
THE OFFER + GUARANTEE LINE	Specific offer + 90-day money-back as confidence frame.	Ad builds desire but ending doesn't harvest it. Clicks drop dramatically.

Recommended Launch Batch (First 4 Weeks)

Below is the specific recommendation for the first launch batch. Built to give Andromeda enough variety to learn fast while staying tactically focused on the highest-probability winners.

#	Angle	Format(s)	ICP	Priority
1	Angle 1: Nurse Refusing Metformin	Long-form VSL + UGC variant	Type-2 Refuser	PROVEN WINNER
2	Angle 5: 30-Day Glucose Diary	Mid-form testimonial + carousel	Newly-Scared Prediabetic	PROVEN WINNER
3	Angle 6: Forgotten Naval Cure	Long-form VSL + storyteller short	Type-2 Refuser, History 50+	PROVEN WINNER
4	Angle 4: Daughter Who Found Something	UGC + reel + static	Caregiver (UNDERSERVED)	HIGH PRIORITY
5	Angle 9: Show Don't Tell	Demonstration short x 3 variants	All ICPs	CHEAP VOLUME
6	Angle 7: I'm Not My Mother	UGC + reflective reel	Post-GD (UNDERSERVED)	HIGH PRIORITY
7	Angle 12: Doctor's Office Moment	UGC + carousel	Newly-Scared Prediabetic	VOLUME TEST
8	Angle 3: Why Cinnamon Didn't Work	UGC + carousel	Newly-Scared Prediabetic	VOLUME TEST
9	Angle 10: Anti-Marketing Confessional	Founder-voice short	Optimization Adult	DIVERSITY TEST
10	Angle 8: After Ozempic	UGC + carousel	Off-Ramper (UNDERSERVED)	FRONTIER TEST

***LAUNCH BATCH SUMMARY:** 10 angles × 1-3 formats each = 18-22 distinct creative units in the launch. Three are proven winners adapted with new variants. Three are high-priority underserved-segment plays. The remaining four are testing volume formats and frontier-segment exploration. This gives Andromeda real variety while concentrating production on highest-probability conversions.*

PART FIVE

EVOLVE

Read the data. Extract patterns. Compound the advantage.

Every batch of ads answers questions whether we're listening or not. EVOLVE makes sure we're listening, then converts what we hear into the next batch. Each cycle should produce more winners at lower cost than the last. After three cycles, we should know more about this category than any competitor.

The Diagnostic Framework

Read the metric. Diagnose the failure. Fix the right thing. Most agencies tweak headlines when the actual problem is in the offer architecture. Below is the metric-to-fix map.

Metric Failure	Diagnosis	Where to fix it
Hook Rate < 25%	Ego activation didn't land. Viewer felt nothing.	Go back to IGNITE. Is the first 2 seconds activating fear, identity, or desire? Or is the ad starting with product, logo, or setup? Test 4-6 different hook openings on the same body.
Hold Rate drops at 5s	Attention earned then lost. The bridge to selling content was too slow.	REFRAME problem. Mechanism is being explained instead of demonstrated. Cut education. Show faster. The Cassia/Ceylon reveal needs to happen by second 8-10 in short-form.
Low CTR on long-watchers	Watching but not clicking.	CTA is generic or offer is unclear. Test identity-driven CTAs. Test offer specificity. 'Try risk-free for 60 days' often outperforms 'Shop now.'
High CTR, low ATC	Click-through but not adding.	Page message match. Does the landing page carry the same Big Idea, the same emotional tone, the same proof beats? Or does the ad open one expectation and the page close a different one?
ATC, no CVR	Adding to cart but not buying.	Offer architecture, pricing, shipping, checkout friction. The ad worked, the page worked, the cart didn't. Test bundle structures, free shipping thresholds, urgency frames.

Metric Failure	Diagnosis	Where to fix it
High refund rate	Customer expectation mismatch.	Ad is overselling. Pull back claim intensity. Make sure post-purchase experience matches pre-purchase promise. High refunds are not a guarantee problem, they're an integrity problem.
Low subscription retention	Customers buy once, don't stick.	Onboarding sequence problem. They aren't seeing or feeling results in time. Add a Day 7, Day 21, Day 60 email sequence with progress check-ins, expectation calibration, and CGM-style tracking encouragement.

Pattern Extraction Questions (After Each Cycle)

After 5-7 days of meaningful spend per concept, sit down with the data and answer these questions. No opinions. No lobbying for favorites. Just data. After three cycles, the answers compound into a database of category-specific intelligence that no competitor can buy.

- Which emotional driver sold best? Fear vs Identity vs Desire vs Status. Track this by ROAS, not CTR.
- Which angle had the highest hook rate? Build more variants through the winning emotional doorway.
- Which format delivered best per ROAS? Static vs UGC vs B-roll vs VSL. Invest production where ROI is concentrated.
- Which hook type stopped the scroll? Confession vs Confrontation vs Contrast vs Demonstration. Pattern across our top 5 hooks.
- Which CTA frame drove ATC? Identity pressure vs Confidence guarantee vs Specific time-bound.
- Which length performed by ICP? Different segments respond to different lengths. Track separately.
- Which sophistication level matched? Stage 3 mechanism ads vs Stage 5 identity ads. Likely different winners by segment.
- Which proof beat resonated most? Bark contrast vs pile shot vs solubility test vs COA reveal.
- Which testimonial archetype outperformed? Nurse vs Daughter vs Self-narrator. Cast more of the winner.
- Which compliance framing held up? Where did Meta flag claims? Adapt language for next cycle.

The Four-Week Production Workflow

This document is the operating system. The workflow below is what runs on it. Repeatable. Compounding. Each cycle gets cheaper, faster, and more profitable than the last.

Week	What gets done
Week 1: STRATEGY	Review last cycle's data. Update angle priorities. Reassign budgets. Pull the next 2-3 angles for production. Refine ICP definitions if needed. Update the Big Idea language if competitors have moved.
Week 2: SCRIPTS + STORYBOARD S	Write scripts using the persuasion stack template. Mark non-negotiables. Build storyboards for visual proof beats. Cast UGC creators for testimonial-format ads. Brief production with one-page docs that clearly distinguish flexible vs locked elements.
Week 3: PRODUCTION	Shoot UGC. Shoot demonstrations. Edit cuts. Build static carousels. Generate variations of every winning hook on text-on-screen format. Build hook-test sets (same body, 6 different opening lines).
Week 4: LAUNCH + LEARN	Launch 10-18 concepts in single Advantage+ campaign with broad targeting. Mix short, mid, and long-form. Run for 5-7 days at meaningful spend per concept. Run diagnostic. Extract patterns. Feed into next cycle's strategy week.

Creative Velocity Targets

- **Concepts per launch:** 10-18 fundamentally different angles or format variants.
- **Refresh cycle:** Every 14 days. New batches launched even before previous ones fatigue.
- **Hook variants per winning concept:** Once a concept proves, generate 5-8 hook variations to extend its life.
- **Format diversification per winning angle:** Once an angle proves, build it in 3-5 different formats to harvest different placements.
- **Production output per cycle:** 20-30 unique ad units (counting hook variants and format ports).
- **Testing budget allocation:** 70% on proven winners, 20% on high-priority new angles, 10% on frontier exploration (underserved ICPs).

APPENDICES

Appendix A: Headline Library by Angle

These are the headline starters production can pull from when scripting any of the twelve angles. Organized by emotional doorway. Each one is built to feed the same Big Idea.

FEAR-DRIVEN HEADLINES

- Most cinnamon supplements are quietly stressing your liver. Here's how to tell.
- If you've taken cinnamon capsules every day for years, this is worth 60 seconds of your time.
- The cinnamon question your supplement label refuses to answer.
- This is what happens when you take counterfeit cinnamon for two years.
- Three reasons your cinnamon supplement is doing nothing for your blood sugar.
- If your morning numbers won't drop, your cinnamon may be the wrong kind.
- Your cinnamon supplement might not be cinnamon. Here's what it is.

IDENTITY-DRIVEN HEADLINES

- I refused metformin for 3 years. Here's exactly what I did instead.
- I'm an ICU nurse. I've watched what diabetes does. I refuse to become my patients.
- My mom developed type 2 at 52. I just got my first prediabetic A1C at 41. I'm intervening.
- I'm a pharmacist. Here's what I take when I don't want my doctor to know.
- I came off Ozempic six months ago. Here's the protocol that's kept me there.
- I'm 41 and prediabetic. I refuse to wait until I'm 51 to start fixing this.
- I tried metformin once. Three days later I threw it away. Here's why.

CURIOSITY/PATTERN INTERRUPT HEADLINES

- In 1803, a Royal Navy doctor figured out something about cinnamon. Then it disappeared.
- Why pure cinnamon dissolved in oil works when cinnamon in capsules doesn't.
- There's a footnote in a 220-year-old medical log that changed how I take cinnamon.
- The Sri Lankan cinnamon harvest takes 11 months. Most supplement brands skip it.
- If your cinnamon doesn't smell like a bakery, it's probably the wrong species.
- Watch what happens when cinnamon meets water vs cinnamon meets oil.

- Same word, different plant. Why most cinnamon capsules are scientifically confused.

DEMONSTRATION-DRIVEN HEADLINES

- [Cold open: 14 generic cinnamon capsules vs 1 Metabolae softgel.] Same dose.
- Two glasses. Same cinnamon. One has water. One has oil. Watch what happens.
- This is what real Ceylon bark looks like. This is what's in most capsules.
- Here's the third-party lab certificate for our latest batch. Most brands don't have one.
- [Day 1: 132. Day 30: 98.] Here's what changed.

CARE-DRIVEN HEADLINES (FOR CAREGIVER ICP)

- My mom refused everything. Doctors, diets, supplements. Then I found this.
- Three things I tried for two years to help my dad. Only one worked.
- If you're trying to help someone you love and they keep refusing, this might be the one they'll take.
- She wouldn't take a single pill for her blood sugar. She takes this every morning.
- How I finally got my mom to actually take her supplement (one softgel, no taste, that's it).

Appendix B: Master Script Templates

Below are skeleton scripts production can use as templates. Each one demonstrates the persuasion stack in a different format.

SAMPLE SCRIPT 1: Short-form UGC (45 seconds)

Format: UGC talking head. ICP: Newly-Scared Prediabetic. Angle: 'Why The Cinnamon You Tried Didn't Work.'

[0:00 — EGO ACTIVATION]

"If you've tried a cinnamon supplement and saw nothing happen, you're not crazy. You were sold the wrong cinnamon."

[0:05 — IDENTITY TENSION]

"I'm 54. My A1C was 6.3 last year. I tried three different cinnamon supplements from three different brands. Watched my numbers like a hawk. Nothing moved. I gave up on the whole ingredient."

[0:18 — MECHANISM]

"Then I learned something. 90% of what's labeled 'cinnamon' in supplements is Cassia, a different species from China. The studies that made cinnamon famous used Ceylon. The capsules I was buying were Cassia. Same word. Different plant. Different result."

[0:30 — PROOF BEAT]

"[Visual: Ceylon bark vs Cassia bark side by side.] You can see the difference yourself. Ceylon is soft, multi-layered, light brown. Cassia is hard, single-layer, dark. Most supplements use the second one because it's cheaper."

[0:38 — OFFER + CTA]

"This one's real Ceylon. Lab tested. 90 days money back. Track your morning numbers and if they don't move, send it back. Worth a try if cinnamon's the only thing you haven't ruled out."

SAMPLE SCRIPT 2: Mid-form Advertorial (2 minutes)

Format: Story-format advertorial / mini-VSL. ICP: Type-2 Refuser. Angle: 'Nurse Who Refused Metformin' (variant).

[0:00 — EGO ACTIVATION]

"My doctor handed me a metformin prescription on a Tuesday. By Friday it was in the trash. Not because I'm reckless. Because I've been a nurse for 32 years and I've watched what that drug does to people who take it forever."

[0:15 — IDENTITY TENSION]

"I'm not anti-medication. If you need it, take it. But I had an A1C of 6.7. Borderline. I had a window. I wanted to use it. I wasn't going to spend the next 30 years on a drug that I'd seen wreck people's stomachs and deplete their B12 and quietly damage their kidneys. There had to be another way. I just had to find it."

[0:35 — DISCOVERY/MECHANISM SETUP]

"For two years I tried everything. Cut carbs. Walked 10,000 steps a day. Apple cider vinegar before meals. Berberine. Cinnamon supplements from three different brands. My A1C dropped from 6.7 to 6.4 and stuck there. Better. Not enough. And I was exhausted. Cranky. Constantly thinking about food."

[0:55 — REFRAME REVEAL]

"Then I read something that changed everything. Most cinnamon supplements aren't even cinnamon — at least not the kind in the studies. They're Cassia. Different species. From China. Way more of a liver-stressing compound called coumarin, way less of the actives that actually do something."

[1:15 — MECHANISM EXPANSION]

"And the few that are real Ceylon are usually underdosed. The studies used 6,000 to 7,000 milligrams a day equivalent. Most capsules have 1,000. And they're dry powder, which barely absorbs because cinnamon's active compound only dissolves in fat. Three failures stacked. I'd been taking the broken version for two years."

[1:30 — PROOF + IMPACT]

"I switched to a real Ceylon extract suspended in MCT oil. Within three weeks, my fasting glucose dropped 38 points. Eight weeks later my A1C was 5.7. My doctor pulled up my chart, looked at me, and asked what I'd changed. I told her. She didn't argue."

[1:50 — OFFER + IDENTITY CTA]

"If your numbers won't budge no matter what you try and you're staring down the metformin conversation, you might be in the same place I was. The right cinnamon, the right dose, the right delivery. Ninety days money-back. If your morning numbers don't move, you pay nothing. The version of you that intervened in time was always going to make a decision about today. This is just which one."

SAMPLE SCRIPT 3: Demonstration Short (20 seconds)

Format: B-roll + deadpan VO. ICP: All. Angle: 'Show Don't Tell' (variant).

[0:00] [Visual: glass of water + spoon. Cinnamon powder added.]

"This is what cinnamon does in water." [Powder floats, doesn't dissolve.]

[0:05] [Visual: glass of MCT oil + spoon. Same cinnamon powder.]

"This is what cinnamon does in oil." [Powder dissolves, integrates.]

[0:10] [Cut to Metabolae softgel.]

"Most cinnamon capsules use dry powder. Yours probably does. Here's why ours doesn't."

[0:15] [Visual: COA close-up.]

"Real Ceylon. Suspended in MCT oil. Lab tested. Try it for 90 days. Refund if your numbers don't move."

Appendix C: Compliance Guardrails

Per direction, the strategy is built at maximum strength and legal review will soften specific lines if needed. Below is the risk-mapping production should reference when scripting.

LOWER RISK (use freely)	MODERATE RISK (use in testimonial frames)	HIGHER RISK (avoid in brand voice)
Supports healthy blood sugar levels	'My fasting glucose dropped from X to Y' (testimonial)	Lowers blood sugar (brand voice claim)
Supports metabolic balance	'My A1C went from X to Y in eight weeks' (testimonial)	Reduces A1C (brand voice claim)
Helps maintain insulin sensitivity already in normal range	'I refused metformin' (testimonial / personal choice)	Replace metformin / replaces medication
Supports healthy circulation	'My doctor said keep doing whatever you're doing' (anecdotal)	Doctor recommended (without specific endorsement)
Antioxidant support	'I felt steadier energy after two weeks' (subjective testimonial)	Cures / treats / prevents diabetes (any framing)
Made with authentic Ceylon cinnamon	'My morning crashes stopped' (subjective testimonial)	Reverses prediabetes / type 2 diabetes
Coumarin-tested	Sleep / energy / focus improvements as anecdote	Specific numerical promises in brand voice (e.g. 'drops glucose by 40 points')

Compliance Operating Rules

- **Testimonial framing carries different exposure than brand voice.** A first-person 'my numbers improved by X' is acknowledged as a personal report. The brand making the same claim about itself is making a drug claim.
- **Always include the FDA disclaimer.** Bottom of every landing page, in every long-form video voiceover at the close. 'These statements have not been evaluated by the Food and Drug Administration. This product is not intended to diagnose, treat, cure, or prevent any disease.'
- **Never put doctor recommendations in their mouth.** 'My doctor said keep doing whatever you're doing' is fine. 'Doctors recommend Metabola' is not. Watch the line in scripts.
- **Numerical claims are higher risk.** Testimonials can be specific. Brand voice should not be.
- **Drug-name proximity is a flag.** Mentioning metformin is fine. Saying 'instead of metformin' or 'replaces metformin' is a drug-replacement claim.

Appendix D: The Scientific Footing

Every claim in this strategy traces to peer-reviewed evidence or to plausible mechanism. Below is the science the team should know cold so they can defend any angle in compliance review.

Cinnamon's Plausible Mechanisms (with Citations)

- Cinnamaldehyde activates GLUT4 translocation, the same pathway exercise activates, allowing glucose to enter cells without insulin signaling (Beheshti et al., 2024).
- Polyphenols in Ceylon cinnamon inhibit α -glucosidase and α -amylase, slowing post-meal carbohydrate absorption (Senevirathne et al., 2022).
- Cinnamon's antioxidant compounds reduce NLRP3 inflammasome activation, addressing the chronic inflammation underlying insulin resistance (Bibi et al., 2024).
- Standardized *Cinnamomum zeylanicum* extract at 1000mg/day showed significant FBS reduction in T2DM patients (Muthukuda et al., 2025, RCT in PLOS ONE).
- Phase I clinical trial of Ceylon cinnamon at 85-500mg showed beneficial anti-hyperlipidaemic and blood pressure effects with no significant adverse events (Ranasinghe et al., 2017).
- Meta-analysis of 16 RCTs showed mean fasting glucose reduction of approximately 9.8 mg/dL with cinnamon supplementation (Deyno et al., 2019).

The Cassia/Ceylon Distinction (Verifiable)

- Cassia (*Cinnamomum cassia*) contains 0.4-12% coumarin by weight. Ceylon (*Cinnamomum verum*) contains 0.004% coumarin or less.
- EU Tolerable Daily Intake for coumarin is 0.1 mg/kg body weight (EFSA 2008). A 70kg adult exceeds this with about 2 tsp of Cassia powder.
- Coumarin is hepatotoxic in susceptible individuals. Banned as flavoring agent in food in most jurisdictions.
- Ceylon and Cassia are botanically different species, not varieties of the same plant.

The MCT Bioavailability Argument

- Cinnamaldehyde is lipophilic. Solubility in water is low; solubility in lipid carriers is significantly higher.
- MCT oil bypasses standard fat digestion and is absorbed via the portal vein, bringing dissolved compounds with it.
- MCT oil independently shows benefit in metabolic health: improved insulin sensitivity, ketogenic-pathway support, satiety enhancement.
- Specific bioavailability studies comparing cinnamon-in-powder vs cinnamon-in-MCT have not been published in human trials. The argument is mechanistic plausibility plus pharmaceutical formulation principle, not direct human data. Treat as 'scientifically reasoned' rather than 'clinically proven.'

The Dose Question

- Most cinnamon RCTs used 1-6 grams per day of powdered bark.
- Concentrated extracts (water/ethanol) at 12:1 ratio represent the bioactive equivalent at lower physical dose.
- 600mg of 12:1 extract = 7,200mg raw bark equivalent in mechanistic activity. This is a standard pharmacology framing.
- Note: 'equivalent' is a marketing-acceptable term but should be flagged in compliance review. Some regulators prefer 'derived from.'

Appendix E: The Metrics Dashboard

These are the metrics to track every cycle. If a number isn't in this list, it's noise.

Metric	Target / Benchmark	What it tells us
Hook Rate (3-sec view rate)	>25% acceptable, >35% strong	Whether the first 2-3 seconds are activating ego. The single most important early-stage signal.
Hold Rate (15-sec view rate)	>15% acceptable, >25% strong	Whether the bridge from hook to mechanism is working. Drops here mean the reframe is too slow.
CTR (link click rate)	>1.5% acceptable, >2.5% strong	Whether the offer + CTA is harvesting the desire built up in the body of the ad.
CPC (cost per click)	<\$1.50 acceptable, <\$0.80 strong	Composite of hook quality and offer relevance. High CPC usually means the algorithm is rate-limiting delivery.
CVR (conversion rate)	>2% acceptable, >4% strong	Whether the landing page and offer architecture close what the ad opened.
AOV (average order value)	>\$45 acceptable, >\$70 strong	Bundle uptake. Subscription conversion. Tells us whether the offer architecture is working.
ROAS (return on ad spend)	>2.0 acceptable, >3.0 strong	The ultimate test. All upstream metrics serve this one.
Subscription rate	>25% acceptable, >40% strong	Whether buyers are committing to the protocol. Indicator of LTV.
Refund rate	<8% acceptable, <4% strong	Mismatch between ad promise and product experience. High = overselling.
Subscription retention (M3)	>50% acceptable, >70% strong	Whether the product delivers enough perceived benefit to keep the customer past the first refill.

Appendix F: Competitive Watch

The category is saturated with lookalikes. Below is the active competitive landscape and the strategic response per competitor type.

Competitor Type	Examples	Our strategic response
Lookalike Amazon brands	Micro Ingredients, LIRAE, Primal Remedies	Win on trust signals, not formula. Sourcing footage. COA reveal. Founder voice. Build brand depth they can't replicate at Amazon-FBA scale.
Legacy supplement brands	NOW Foods, Nature's Bounty	Acknowledge their trust. Don't attack. Win by being the modern, mechanism-explained, transparent alternative for buyers who outgrew commodity brands.
Premium clinical brands	Doctor's Best CinSulin, Pure Encapsulations	Own the popular-direct-response space they don't want to enter. Faster creative cycles, sharper voice, ICP-specific creative they don't make.
Cassia-based generics	Walmart house brands, drugstore generics	Use them as the implicit villain in the counterfeit-cinnamon reframe. Don't name them, indict the category.
DTC-funnel competitors	Other 7200mg/MCT funnel sites	Out-spend, out-volume, out-evolve. They have the same formula. We have the better creative system. The gap widens every cycle.

The Strategic Mandate

This document is not a deck. It is the operating system Metabolae will run on for twelve months.

If we execute the angle library at the production volume Andromeda needs, if we evolve the patterns each cycle, if we build the underserved-segment plays that no competitor is making, the brand will own the Ceylon cinnamon category by Q4 of next year.

The win condition is not a single hero ad. The win condition is a creative production system that compounds, a mechanism story so well-developed that competitors are forced to copy us (and arrive too late), and a brand voice so distinct that 'real Ceylon' becomes synonymous with Metabolae the way 'cyclonic suction' became synonymous with Dyson.

The math is simple. The work is not. Let's get to it.

DISCOVER. REFRAME. IGNITE. VARIATE. EVOLVE.

End of document.